

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Metronidazole gel or Azelaic acid for the treatment of Rosacea

Summary of NICE / NICE CKS Guidelines for Rosacea

Overview

- **Rosacea** is a chronic, relapsing skin condition that affects the **central face**, characterised by **flushing, persistent erythema, papules/pustules**, and sometimes **telangiectasia, eye symptoms**, or **skin thickening** (phymatous rosacea).
- Most common in **fair-skinned adults aged 30–60**, especially women, though **rhinophyma** is more common in men.

Subtypes of Rosacea

1. **Erythematotelangiectatic** – persistent redness, flushing, visible vessels.
2. **Papulopustular** – acne-like spots on red background.
3. **Phymatous** – thickened skin (e.g. rhinophyma).
4. **Ocular** – dry, sore, gritty eyes, blepharitis.

Many patients have overlapping features.

Assessment

- Diagnosis is clinical based on symptom pattern and distribution.
- Rule out other conditions (e.g. acne, seborrhoeic dermatitis, lupus).
- Ask about **triggers**: sun exposure, alcohol, spicy food, stress, temperature extremes, skin products.

Management

1. General Measures

- Identify and **avoid triggers**.
- Use **non-irritant, non-comedogenic skincare**.
- Daily use of **broad-spectrum sunscreen (SPF 30+)**.
- Avoid steroid creams unless prescribed short-term by a specialist.

2. Pharmacological Treatments (Tailored to Symptoms)

Symptom	First-Line Treatment	Notes
Erythema/flushing	Brimonidine gel (0.33%)	Vasoconstrictor; apply once daily
Papules/pustules	• Topical metronidazole 0.75%/1% • Topical azelaic acid 15–20% • Topical ivermectin 1% Apply once or twice daily for up to 8–12 weeks Moderate-severe or unresponsive papulopustular rosacea Oral tetracyclines (e.g. doxycycline 100 mg OD) Use for 6–12 weeks Ocular rosacea • Artificial tears • Oral doxycycline (if moderate/severe) Refer to ophthalmology if severe or vision is affected	

3. Specialist Referral

Refer to dermatology if:

- Diagnosis is uncertain
- Severe or treatment-resistant disease
- Phymatous rosacea (may require surgery/laser)
- Significant **psychosocial impact**

Follow-Up

- Review after **8–12 weeks** of treatment.
- Taper or stop treatment if symptoms are controlled.
- Long-term maintenance with topical agents may be needed.

Patient Group Direction

for the supply/administration of

Metronidazole gel

for the treatment of

Rosacea

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Metronidazole gel is indicated for the topical treatment of rosacea in adults, particularly for reducing inflammatory lesions such as papules and pustules.
Inclusion criteria	- Adults aged 18 years and over. - Diagnosis of mild to moderate rosacea with inflammatory lesions. - Informed consent obtained. - No contraindications to metronidazole.
Exclusion criteria	- Known hypersensitivity to metronidazole or other nitroimidazoles. - Severe rosacea requiring systemic treatment. - Broken, irritated, or eczematous skin. - Pregnancy or breastfeeding unless deemed appropriate by prescriber.
Cautions	- Avoid contact with eyes, mucous membranes, and broken skin. - Use sunscreen and avoid excessive sunlight exposure. - Wash hands after application. - Monitor for prolonged use; review at 8–12 weeks for effectiveness.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Metronidazole gel 0.75% w/w
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical application to affected areas of the face.
Dose and frequency	Apply a thin layer to the affected area twice daily.
Quantity to be administered and/or supplied	Up to 1 x 30 g tube per treatment course.
Maximum or minimum treatment period	Initial course of up to 8 weeks; reassess continued use beyond 12 weeks.
Adverse effects	Dryness, skin irritation, stinging, redness, metallic taste. Rare: peripheral neuropathy with prolonged use. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	- To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. - Advise on the importance of avoiding trigger factors wherever possible. - Suggest that a diary may be helpful to identify stimuli and triggers that may exacerbate rosacea. - Advise on the importance of effective sun protection and to avoid the use of sunbeds. - Advise on the use of regular non-oily emollients if the skin is dry and the possible use of yellow- or green-tinted cosmetics to help camouflage skin erythema.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)

● Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
13/12/25		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		13/12/2025
Chris Pilkington	Pharmacist GPhC: 2046322		13/12/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	13 th December 2025

Patient Group Direction

for the supply/administration of

Azelaic Acid

for the treatment of

Rosacea

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Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Azelaic acid is indicated for the topical treatment of mild to moderate papulopustular rosacea in adults.
Inclusion criteria	- Adults aged 18 years and over. - Diagnosis of papulopustular rosacea suitable for topical therapy. - Informed consent obtained. - No contraindications to azelaic acid.
Exclusion criteria	- Known hypersensitivity to azelaic acid or any of the excipients. - Severe rosacea requiring systemic treatment. - Broken, irritated, or eczematous facial skin. - Pregnancy or breastfeeding unless assessed as appropriate by the prescriber.
Cautions	- Avoid contact with eyes, mouth, mucous membranes, and broken skin. - Wash hands after use. - Use sunscreen and avoid excessive UV exposure. - Monitor for skin irritation; discontinue if severe or persistent. - Worsening of asthma in patients treated with azelaic acid has been reported.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Azelaic acid 15% gel
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical application to affected areas of the face. Occlusive dressing or wrappings should not be used, and hands should be washed after applying the gel
Dose and frequency	Apply a thin layer to the affected area twice daily (morning and evening). 2.5cm (1 inch) of gel is sufficient for the entire facial area. In the event of skin irritation, the amount of gel per application should be reduced or the frequency of use should be reduced to once a day until the irritation ceases. If required, the treatment should be temporarily interrupted for a few days.
Quantity to be administered and/or supplied	Up to 1 x 30 g tube per treatment course.
Maximum or minimum treatment period	Initial course of up to 12 weeks; reassess ongoing need periodically. The duration of use can vary from person to person and also depends on the severity of the skin disorder. In general, a distinct improvement

	becomes apparent after 4 weeks of treatment. To obtain optimum results, Azelaic Gel can be used over several months in accordance with the clinical outcome. If there is no improvement after 2 months or a new exacerbation of rosacea, Azelaic Gel should be discontinued, and other therapeutic options should be considered.
Adverse effects	Burning, stinging, itching, dryness or redness at application site. Rare: hypopigmentation or hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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